

Health Policy in Switzerland

Course handout

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Course overview

This two-day course provides a practical introduction to Swiss health policy. It examines how institutions, actors and political processes shape health policy decisions and implementation.

The course moves from understanding the system to analysing concrete health-policy challenges and, finally, seeing policy in practice.

Course programme

1. Analytical toolkit

How does Swiss health policy work?

- Institutions and responsibilities
- Actors and interests
- Political process

2. Five big questions

How do we apply the toolkit to major challenges?

- Understand the problem
- Analyse the politics
- Develop policy solutions

3. Policy in practice

How does it work in the real world?

- Canton of Bern
- Federal Palace
- Exchange with policymakers

Overarching question

Why is solving major health-policy problems in Switzerland so politically difficult, and under what conditions can change nevertheless happen?

From policy ideas to political reform

Recurring questions in Swiss health policy:

- **Who can decide?** Which level of government is responsible, or is it none of the state's business?
- **Who pays, who gains, who loses?** How are costs, benefits, access, autonomy and authority distributed?
- **Who can shape the reform?** Who can support, modify or block it?
- **How can change happen?** What path through the political process is possible?
- **Who makes it work?** Who is responsible for implementation, and how do we know whether it works?

A good policy idea is not yet a politically feasible and workable reform.

Part I: Analytical toolkit

Three questions

We will use three questions throughout the course:

1. **Who can decide?**
Institutions and responsibilities
2. **Who wants what?**
Actors, interests and political conflict
3. **How does change happen?**
The Swiss policy process

1. Who can decide?

Federalism

Political power is shared across levels of government

“Switzerland is a federal state. In practice this means that political power is shared by the Confederation, the cantons and the communes.”

Self-rule

Cantons retain substantial **autonomy and responsibilities** within their territory.

Shared rule

Cantons also **participate in decision-making at the federal level**.

Federalism therefore combines **cantonal autonomy** with **participation in the federation**.

Source: Swiss Confederation, ch.ch, “Federalism”.

How did Switzerland become a federal state?

Switzerland was built from below

Before 1848, Switzerland was a confederation of largely sovereign cantons.

The Federal Constitution of 1848 created a federal state, but the cantons retained substantial autonomy.

Art. 3: The cantons are sovereign insofar as their sovereignty is not limited by the Federal Constitution.

Federal authority is therefore not the default: powers must be **assigned to the Confederation by the Constitution**.

Source: Federal Constitution, Art. 3.

Health policy: gradual centralisation

Federal involvement in health policy expanded incrementally.

1848: Federal state

Health remains predominantly cantonal. The Confederation receives only a limited competence for epidemic control.

1890: Health insurance

A constitutional basis is created for federal legislation on sickness and accident insurance.

1911: KUVG

The first federal sickness and accident insurance law is adopted. Sickness insurance remains voluntary.

1994 / 1996: KVG

Mandatory basic health insurance is introduced, together with a stronger national regulatory framework.

Today

Federal responsibilities have expanded, but Switzerland still has no general federal competence for healthcare provision.

Swiss health policy has become more centralised, but not fully centralised.

Subsidiarity

Responsibilities should remain as close as possible to those affected

Art. 5a: The principle of subsidiarity must be observed in the allocation and performance of state tasks.

The Confederation should undertake tasks that require action at the federal level, while other responsibilities remain at lower levels.

In health policy, responsibilities may therefore remain with:

- cantons
- municipalities
- organisations close to service delivery

Federalism asks who can decide. Subsidiarity asks how high up a task needs to go.

Source: Federal Constitution, Arts. 5a and 43a.

Economic freedom

And not every problem is the state's business

Art. 27: Economic freedom is guaranteed.

It includes the freedom to choose an occupation and to pursue a private economic activity.

In health policy, this contributes to a strong role for:

- private providers
- health insurers
- professional autonomy
- negotiated and self-regulatory solutions

The question is therefore not only **which level of government should act**, but also **whether and how the state should intervene at all**.

Source: Federal Constitution, Art. 27; Trein, Rüefli & Vatter (2023).

Direct democracy

Citizens are directly involved in major political decisions.

Referendum

A referendum allows citizens to challenge decisions taken by parliament.

The possibility of a referendum matters even when no vote ultimately takes place: policymakers anticipate potential opposition and may adapt proposals accordingly.

Popular initiative

Popular initiatives allow citizens to propose constitutional change and can place new issues on the political agenda.

Health policy at the ballot box

Examples include:

- 1899: federal sickness and accident insurance law rejected
- 1994: KVG accepted
- 2012: managed care reform rejected
- 2014: single-payer initiative rejected
- 2021: Nursing Initiative accepted
- 2024: EFAS accepted

Citizens can both put health-policy issues on the agenda and act as veto players.

Power sharing and negotiated governance

Swiss policymaking is characterised by broad inclusion, negotiation and compromise.

This includes:

- consultation with affected actors
- coordination between Confederation and cantons
- negotiation with organised interests
- attempts to build broad political coalitions

Opposition is therefore often incorporated into policymaking before the final decision.

Corporatism

Corporatism is a specific form of negotiated governance in which organised interests participate directly in policy formulation, regulation or implementation.

Health policy provides important examples, particularly where providers and insurers negotiate rules or tariffs that are subsequently approved or regulated by the state.

Public authority and private self-regulation are often intertwined.

Who can decide? Summary

Three institutional characteristics are particularly important:

1. Federalism

Authority is divided across levels of government.

2. Direct democracy

Citizens can place issues on the agenda and block reforms.

3. Negotiated governance

Political decisions frequently emerge from bargaining and compromise.

But institutions alone do not explain policy outcomes.

We also need to know **who the relevant actors are and what they want.**

2. Who wants what?

Public and private actors

Swiss health policy involves a large number of public and private actors.

Public actors

- Confederation
- cantons
- municipalities
- parliament
- political parties

They regulate, finance, plan and implement health policy.

Private and organised actors

- healthcare providers
- professional organisations
- health insurers
- pharmaceutical and other industries
- patients and civil society

They provide services, finance healthcare, negotiate rules and attempt to influence policy.

Swiss health policy emerges from the interaction of public authority and organised societal interests.

Public actors

Confederation

The Confederation has important responsibilities in areas including health insurance, pharmaceuticals, public-health protection and regulation.

Cantons

Cantons retain major responsibilities for healthcare provision, planning, implementation and financing.

Municipalities

Municipal responsibilities vary across cantons and may include areas such as long-term care and local health-care provision.

The state is not one actor. Different levels of government may have different responsibilities and interests.

Private and organised actors

Important organised actors include:

Providers and professions

Medical associations, hospitals, nursing organisations and other health professions.

Insurers and industry

Health insurers, pharmaceutical companies and other health-sector organisations.

Patients and civil society

Patient organisations, advocacy groups and other civil-society actors.

These actors differ in their **interests, resources, organisation and access to policymakers.**

Citizens

Citizens occupy several roles simultaneously:

- patients
- premium payers

- taxpayers
- voters

These roles can generate competing preferences.

A person may, for example, favour better access to healthcare while also opposing higher premiums or taxes.

What do actors want?

Health-policy reforms can redistribute:

- **money**
- **access**
- **autonomy**
- **authority**

Interests are not necessarily purely financial.

They may reflect:

- material interests
- organisational interests
- professional ideas
- political values

Who has influence?

Political influence can derive from different resources:

- formal authority
- expertise and information
- implementation capacity
- organisational resources
- access to decision-makers
- mobilisation capacity
- referendum capacity

Formal responsibility and political influence are not the same thing.

Who wants what? Summary

When analysing a reform, ask:

1. Who are the main actors?
2. What do they want, and why?
3. Who gains and who loses?
4. What political resources do they possess?
5. Who can support, modify or block the proposal?

3. How does change happen?

The policy cycle

A simple analytical map distinguishes five stages:

1. **Agenda**
2. **Formulation**
3. **Decision**
4. **Implementation**
5. **Evaluation**

The policy cycle is not a description of a perfectly linear process.

Reforms can move backwards, stall, change direction or develop differently during implementation.

The Swiss policy process

Agenda

How does an issue become recognised as requiring political action?

Formulation

Which solutions are developed, and by whom?

Consultation

How do cantons, parties and organised interests react to the proposal?

Decision

Can a majority be assembled in government and parliament?

Direct democracy

Is the decision challenged through a referendum, or is the issue shaped by an initiative?

Implementation

How is the policy translated into practice?

Evaluation

What effects does the reform produce?

A reform can be changed, delayed or blocked at multiple points.

Consultation

The consultation procedure (*Vernehmlassung*) gives affected actors an opportunity to respond to proposed legislation.

Cantons, parties and organised interests can:

- support a proposal
- oppose it
- identify implementation problems
- demand changes

Opposition can therefore reshape a policy **before the formal parliamentary decision**.

The long shadow of direct democracy

Direct democracy matters even when there is no referendum.

Policymakers may anticipate a potential referendum and therefore:

- consult affected actors
- modify proposals
- build broader coalitions
- seek compromise

Popular initiatives can also force issues onto the political agenda.

Adoption is not the end

Implementation can involve:

- cantonal authorities
- administrative rules
- financing decisions
- negotiations among stakeholders
- healthcare organisations
- professional practice

A policy adopted in Bern may therefore look different when implemented in practice.

Why now?

Major policy change does not occur simply because a problem exists.

A window for change is more likely when three elements come together:

Problem

An issue is recognised as requiring political attention.

Policy

A viable solution is available.

Politics

The political environment is favourable to action.

Policy entrepreneurs can help connect these elements when a **policy window** opens.

Follow the reform

To understand a reform, reconstruct its trajectory:

Problem → Proposal → Politics → Decision → Implementation → Outcome

Ask why the reform:

- succeeded
- failed
- stalled
- was modified
- produced unexpected consequences

Part II: Five big questions

The analytical toolkit will now be applied to five major challenges in Swiss health policy.

1. How can Switzerland contain healthcare costs?

Healthcare expenditure has increased over time, while cost-containment measures can impose concentrated losses on providers, insurers, patients or other actors.

The political challenge is therefore:

How can Switzerland constrain expenditure growth when cost-saving measures impose losses on powerful actors?

Possible reform cases include:

- cost-containment packages
- tariff reforms
- cost targets
- limits on ambulatory service volumes or billing
- other measures affecting prices or utilisation

Questions for analysis

- What drives the problem?
- Who has responsibility for addressing it?
- Who would gain or lose from the proposed reform?
- Who can support or block it?
- How would it be implemented?

2. How should the costs of healthcare be distributed?

Healthcare financing is not only about the total amount spent.

It also concerns how financial burdens are distributed among:

- households
- taxpayers
- cantons
- insurers
- employers

The political challenge is therefore:

Who should pay for healthcare, and how much solidarity should the system provide?

Possible reform cases include:

- premium subsidies
- the Premium Relief Initiative and counterproposal
- EFAS
- alternative premium models

Questions for analysis

- Who currently bears the costs?
- What distributional problem is the reform intended to address?
- Who gains and who loses?
- Which level of government bears responsibility?
- What political coalition could support change?

3. How can Switzerland secure access to healthcare with a constrained workforce?

Workforce shortages can affect access to healthcare and create substantial regional differences.

The issue concerns not only the number of health professionals, but also:

- retention
- working conditions
- distribution across regions
- professional roles
- organisation of care

The political challenge is:

How can Switzerland maintain access to healthcare when workforce capacity is constrained?

Possible reform cases include:

- implementation of the Nursing Initiative
- measures to strengthen primary care
- medical study places
- practice-assistant programmes
- advanced practice roles
- regional health centres

Questions for analysis

- What exactly is the access problem?
- Which professions and regions are affected?
- Who can intervene?
- Which policy instruments are available?
- What are the political and implementation constraints?

4. How can Switzerland strengthen public health?

Many important determinants of health lie outside individual medical care.

Public-health policy can involve:

- prevention and health promotion
- regulation of health risks
- health literacy
- population-based programmes
- surveillance and health protection
- action on social and environmental determinants

Stronger public-health action can generate conflict over:

- individual freedom
- economic interests
- federal and cantonal responsibilities
- regulation
- long-term versus immediate benefits

The political challenge is:

How can Switzerland strengthen population health when responsibility is fragmented and many interventions reach beyond healthcare itself?

Possible reform cases include:

- the failed Prevention Act
- tobacco advertising
- alcohol or nutrition policy
- national prevention strategies
- health-promotion programmes

5. Can Swiss healthcare become simpler without creating more regulation?

Swiss healthcare combines decentralised responsibilities, multiple payers, large numbers of providers and extensive regulation.

Attempts to solve coordination and quality problems can themselves create additional administrative requirements.

The political challenge is:

Can administrative complexity be reduced without simply adding another layer of regulation?

Possible reform cases include:

- measures to reduce administrative burden
- digitalisation
- the electronic patient record
- quality regulation
- simplification measures in primary care

Your policy challenge

Each group becomes the expert team for one of the five challenges.

Your task is to move from a broad policy problem to a **specific political analysis**.

1. Understand

- What is the central problem?
- How did it develop?
- What is the current situation?
- What evidence shows that it matters?

2. Explain

- Who can decide?
- Who wants what?
- Who gains and who loses?
- Why is addressing the problem politically difficult?

3. Follow a reform

Select one concrete policy proposal.

Reconstruct:

Problem → Proposal → Politics → Decision → Implementation → Outcome

4. Assess

- Why did the reform succeed, fail, stall or change?
- What are its main opportunities and limitations?
- What should happen next?

Policy Challenge Dossier

Prepare a concise two-page dossier.

Page 1: The challenge

Include:

- the challenge in one sentence
- two or three important turning points
- two or three key indicators
- the central political trade-off

Actor analysis

Actor	Responsibility	Interest / position	Influence
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Page 2: The reform

Identify:

- the reform selected
- the main stages of the political process
- key actors and conflicts
- opportunities and limitations
- why it succeeded, failed, stalled or changed
- what should happen next
- two or three unresolved questions

Part III: Policy in practice

Day 2

From policy analysis to political questions

Day 2 uses the analytical toolkit to move from understanding a policy problem to assessing concrete policy proposals.

We return to three questions:

1. **Who can decide?**
Institutions and responsibilities
2. **Who wants what?**
Actors, interests and political influence
3. **How does change happen?**
Political process and implementation

Today's goal

For your policy challenge:

1. update your diagnosis
2. identify one or two concrete policy proposals
3. assess their political opportunities and obstacles
4. test your analysis against the cantonal perspective
5. develop targeted questions for policymakers

Canton of Bern

Test your analysis

During the visit to the Health Office of the Canton of Bern, listen for evidence that:

- confirms your assumptions
- challenges your assumptions
- reveals an implementation problem
- changes your assessment of the policy proposal

After the visit, ask:

What did we learn from the cantonal perspective?

Stress-test your policy proposal

For your selected proposal, ask:

Who can decide?

Which level of government or other actor has authority?

Who pays, gains or loses?

What are the main distributive consequences?

Who supports or opposes it?

What are the principal political coalitions?

Where could it be modified or blocked?

At which stage of the political process is the proposal vulnerable?

Who implements it?

What happens after formal adoption?

How would we know whether it works?

What would constitute policy success?

From analysis to interview questions

The aim is not to ask policymakers for general opinions.

Use your research to identify:

- a specific political choice
- a genuine trade-off

- an unresolved conflict
- an implementation obstacle

A strong interview question:

- concerns a **concrete policy proposal**
- identifies a **real political trade-off**
- can be answered by a national policymaker
- requires a position or explanation
- allows a targeted follow-up

Ask like an interviewer

Too broad

What should Switzerland do about the shortage of healthcare workers?

More targeted

The Nursing Initiative was accepted in 2021. Which measure in its implementation will make the biggest difference to retaining nurses?

Follow-up

If working conditions do not improve, should the Confederation impose stronger requirements on employers?

The research may be complex. **The question should be short.**

Prepare your interview guide

Each group prepares two candidate questions.

For each question, specify:

Element	Preparation
Policy proposal	What concrete measure are we discussing?
What we know	What does our research tell us?
Political tension	What is contested or unresolved?
Main question	Short and direct
Likely answer	What response do we expect?
Follow-up	What will we ask if the central issue is avoided?

Conversation with National Councillor Manuela Weichelt

Our questions will be organised around the five policy challenges.

The objective is a **focused policy conversation informed by evidence**, not five separate presentations.

Use follow-up questions where appropriate.

Final synthesis

Return to our overarching question:

Why is solving major health-policy problems in Switzerland so politically difficult, and under what conditions can change nevertheless happen?

Consider the interaction of:

- institutions
- actors and interests
- political processes
- implementation

Required reading

Trein, Philipp, Christian Rüefli, and Adrian Vatter. 2023. "Health Policy." In *The Oxford Handbook of Swiss Politics*, 714–732. Oxford University Press.

Useful links

Federal and cantonal health policy and regulation

- Federal Office of Public Health (FOPH/BAG)
- Federal Department of Home Affairs (FDHA/EDI)
- Fedlex
- GDK/CDS
- Swissmedic
- Health Promotion Switzerland

Health insurance, financing and tariffs

- prio.swiss
- Common Institution under the KVG
- SwissDRG
- OAAT

Providers, health professions and industry

- FMH
- H+
- alliance care
- Federation of Swiss Psychologists (FSP)
- Physioswiss
- pharmaSuisse
- Interpharma
- Spitex Switzerland
- ARTISET
- OdASanté

Health statistics, monitoring and quality

- Federal Statistical Office (FSO/BFS)
- Swiss Health Observatory (Obsan)
- Federal Quality Commission (FQC/EQK)
- ANQ

International and comparative information

- European Observatory: Switzerland
- OECD Health
- WHO Switzerland
- Commonwealth Fund: Switzerland

Bibliography